BASELINE → TREATMENT → FOLLOW-UP

Your testing and review plan

A reference cycle is not a personalised calendar. Tests listed as symptom-triggered or individual are not presented as mandatory tests at every visit. No universal imaging or tumour-marker interval is invented.

Check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Safety tests and symptoms	Confirm BRAF V600, blood count, kidney/liver tests, medicine interactions and skin assessment. Current Zelboraf product information calls for a baseline electrocardiogram (ECG) and electrolytes including magnesium for all patients; this supplements the National Cancer Control Programme (NCCP) risk-based ECG row. Staging scans establish the sites of melanoma.	Blood count, kidney and liver tests before each cycle. NCCP specifies ECG every four weeks for the first 12 weeks, then every 12 weeks and after dose changes. Its skin-review row states week eight then every 12 weeks; current Zelboraf product information instead requires monthly prescriber skin examinations. The team should supply a reconciled written skin-check plan using the current product information.	Contact the oncology team promptly for fainting or palpitations, eye pain or blurred vision, a blistering or rapidly spreading rash, severe sunburn, persistent vomiting or diarrhoea, reduced urine or yellow skin or eyes. Report new or changing skin growths even when the melanoma is responding.	Current Zelboraf product information calls for monthly skin checks during treatment and for up to six months after stopping, or until another anticancer therapy starts. Head-and-neck checks every three months and chest computed tomography (CT) every six months during treatment screen for other cancers; these are distinct from the individual melanoma-response scan plan. Anal/pelvic checks are considered at baseline/end and when indicated. Record the team's coordinated surveillance plan rather than duplicating scans.	Do not restart held treatment until the team confirms the medicine, dose and timing. Safety tests and unresolved symptoms are reviewed first.
Scans and tumour results	Keep the pathology, stage and relevant biomarker report.	Response scans follow the individual disease plan, separate from safety bloods.	Report new symptoms rather than waiting for a scan.	Ask which scan, its planned date and who will explain the result.	A treatment hold or stop may change the next scan or follow-up date.
Written plan and contacts	Record actual oncology and out-of-hours numbers; examples are not working contact details.	Bring your medicine list and record the dose actually given or taken.	Keep the wallet card and written urgent-symptom plan available.	Ask for a printed updated plan when treatment changes.	Keep the last-dose date and any steroid, hormone or other follow-up instructions.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00102 v6: current regimen source · EMA Zelboraf product information

THE WHOLE TREATMENT JOURNEY

Vemurafenib alone — advanced melanoma: treatment and checks

Advanced melanoma

Vemurafenib twice daily, about 12 hours apart, every day. A 28-day review cycle does not mean a planned tablet-free week.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Safety tests and symptoms NCCP 00102	Confirm BRAF V600, blood count, kidney/liver tests, medicine interactions and skin assessment. Current Zelboraf product information calls for a baseline ECG and electrolytes including magnesium for all patients; this supplements the NCCP risk-based ECG row. Staging scans establish the sites of melanoma.	Blood count, kidney and liver tests before each cycle. NCCP specifies ECG every four weeks for the first 12 weeks, then every 12 weeks and after dose changes. Its skin-review row states week eight then every 12 weeks; current Zelboraf product information instead requires monthly prescriber skin examinations. The team should supply a reconciled written skin-check plan using the current product information.	Contact the oncology team promptly for fainting or palpitations, eye pain or blurred vision, a blistering or rapidly spreading rash, severe sunburn, persistent vomiting or diarrhoea, reduced urine or yellow skin or eyes. Report new or changing skin growths even when the melanoma is responding.	Current Zelboraf product information calls for monthly skin checks during treatment and for up to six months after stopping, or until another anticancer therapy starts. Head-and-neck checks every three months and chest CT every six months during treatment screen for other cancers; these are distinct from the individual melanoma-response scan plan. Anal/pelvic checks are considered at baseline/end and when indicated. Record the team's coordinated surveillance plan rather than duplicating scans.	Do not restart held treatment until the team confirms the medicine, dose and timing. Safety tests and unresolved symptoms are reviewed first.
Scans and tumour results NCCP 00102	Keep the pathology, stage and relevant biomarker report.	Response scans follow the individual disease plan, separate from safety bloods.	Report new symptoms rather than waiting for a scan.	Ask which scan, its planned date and who will explain the result.	A treatment hold or stop may change the next scan or follow-up date.
Written plan and contacts NCCP 00102	Record actual oncology and out-of-hours numbers; examples are not working contact details.	Bring your medicine list and record the dose actually given or taken.	Keep the wallet card and written urgent-symptom plan available.	Ask for a printed updated plan when treatment changes.	Keep the last-dose date and any steroid, hormone or other follow-up instructions.

What each check is for

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Ongoing oral treatment

28-day reference cycle

To control advanced BRAF V600-mutated melanoma with a daily oral targeted medicine; benefit and tolerability are reviewed for the individual.

Ask / **act:** Confirm the current phase, prescription, tests and support medicines.

Sources: NCCP 00102

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-06 · Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.42